

Can Exercise Lower Cholesterol?

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If you've been told your cholesterol is high, you've probably heard the usual advice: eat well, exercise more and maintain a healthy weight.

But can exercise actually change your cholesterol levels?

Yes — regular physical activity can help improve your cholesterol profile and, more importantly, reduce your overall cardiovascular risk.

However, cholesterol isn't just one number, and the way exercise affects it is a little more interesting than simply making your “cholesterol go down.”

First, what actually is cholesterol?

Cholesterol and triglycerides aren't inherently bad — **your body actually needs them**. The problem occurs when certain types are present at unhealthy levels.

Here's a simple way to think about them:

LDL cholesterol – the delivery truck

LDL carries cholesterol from your liver to cells around your body. Your cells use cholesterol to help build cell membranes and make important substances such as hormones. However, when there is too much LDL circulating in your blood, cholesterol can build up within artery walls over time. This is why LDL is often referred to as “bad” cholesterol.

HDL cholesterol – the clean-up crew

HDL helps collect excess cholesterol and transport it back to the liver, where it can be processed and removed from the body. This is why HDL is commonly referred to as “good” cholesterol.

Triglycerides – stored energy

Triglycerides are a type of fat your body uses to store energy. When you eat more energy than your body needs immediately, some of that excess energy can be converted into triglycerides and stored in fat cells to use later. We all need triglycerides, but consistently high levels in the bloodstream are associated with increased cardiovascular risk.

So, rather than thinking of LDL and triglycerides as “bad things” that shouldn't be in your body, think of them as **useful substances that can become problematic when their levels are too high**.

Your GP will usually look at these results together alongside factors such as blood pressure, smoking, diabetes, age and family history to understand your overall cardiovascular risk.

So, how does exercise help?

Regular exercise can positively influence your blood lipid profile in several ways.

1. Exercise can improve triglycerides

One of the more consistent effects of regular physical activity is a reduction in triglyceride levels.

Your muscles require energy when you exercise. Regular activity improves your body's ability to use fats and carbohydrates for energy, which can contribute to healthier triglyceride levels over time.

2. Exercise can help increase HDL cholesterol

When you exercise regularly, your body becomes better at breaking down and using triglycerides for energy. As these triglyceride-rich particles are processed, some of their components are transferred to HDL, helping HDL particles grow and mature. HDL can then act as the body's "clean-up crew", helping transport excess cholesterol back to the liver for processing and removal.

Over time, regular exercise can therefore help increase HDL levels and improve how effectively HDL does its job.

3. Exercise may help improve LDL cholesterol

Exercise can help your body manage LDL more effectively, even if the change in your overall LDL number is sometimes quite small.

Regular exercise can improve your liver's ability to remove LDL particles from the bloodstream. It may also help shift your LDL profile towards **larger, less harmful particles**, rather than smaller, denser particles that are more likely to enter artery walls and contribute to plaque build-up.

So, even if your LDL number doesn't dramatically decrease, **exercise can still improve the way your body handles cholesterol and contribute to a lower overall cardiovascular risk.**

This is why we shouldn't judge the benefits of exercise based on a single cholesterol number.

What type of exercise is best for cholesterol?

You don't need to become a marathon runner.

A combination of **aerobic exercise and resistance training** is generally a great place to start.

Aerobic exercise could include:

- Brisk walking
- Cycling
- Swimming
- Jogging
- Dancing
- Group fitness
- Using a cross-trainer or rowing machine

Resistance training could include gym-based weights, resistance bands, machines or appropriately challenging bodyweight exercises.

Australian physical activity guidelines generally encourage adults to accumulate **150–300 minutes of moderate-intensity activity or 75–150 minutes of vigorous activity each week**, alongside muscle-strengthening activities on at least two days per week.

However, you don't need to immediately achieve those numbers to benefit.

If you're currently inactive, starting with a manageable amount and gradually building your activity can be much more sustainable.

How hard should you exercise?

For moderate-intensity aerobic exercise, think about an effort where you're breathing harder than usual but can still speak in sentences.

A brisk walk may be enough for one person, while someone with a higher level of fitness may need to walk uphill, cycle or jog to achieve a similar intensity.

That's why exercise recommendations aren't one-size-fits-all.

Your current fitness, medical history, medications, symptoms and goals all influence what an appropriate exercise program looks like.

What about weight loss?

Exercise can improve your cardiovascular health even if the number on the scales doesn't change dramatically.

For people who are carrying excess body fat, changes in body composition and weight can sometimes further improve cholesterol and triglyceride levels. But **weight loss isn't the only reason exercise is beneficial**.

Your heart, blood vessels, muscles and metabolic health can all benefit from becoming more physically active.

That's an important reason to look beyond the scales when measuring progress.

Can exercise replace cholesterol medication?

Not necessarily.

Lifestyle changes such as exercise, nutrition and smoking cessation can play an important role in managing cardiovascular risk. However, some people will still require cholesterol-lowering medication.

This can depend on your cholesterol levels, medical history, family history and overall cardiovascular risk.

If you've been prescribed medication, don't stop taking it because you've started exercising. Medication changes should always be discussed with your doctor.

Think of exercise and medication as different tools that can sometimes be used **together**, rather than one automatically replacing the other.

You don't have to wait until you're fit to start

If you've recently been told you have high cholesterol, it's easy to feel like you suddenly need to overhaul your entire lifestyle.

You don't.

Start with what is achievable.

That could be a 10-minute walk after dinner, cycling a few times per week, completing two strength sessions, or simply breaking up long periods of sitting.

Once that becomes manageable, you can build from there.

An **Accredited Exercise Physiologist (AEP)** can help develop an individualised exercise program based on your cardiovascular risk factors, health conditions, current fitness and goals.

At **Valen Health**, we can help you find an appropriate starting point and progressively build your exercise capacity so that movement becomes a realistic part of managing your long-term health.

You don't need a perfect exercise routine to improve your heart health. You just need somewhere to start.